

MP-101: Advanced Imaging: MRI of the Lumbar Spine

Bellcourt Medical Policy | Version 1 | Status: RETIRED (superseded 2025-12-31) | Effective for dates of service 2024-01-01 through 2025-12-31

Field	Value
Policy number	MP-101
Service code(s)	BHA-IMG-0721
Applies to	All clients administered by Bellcourt unless the client's plan document or delegation addendum states otherwise
Owner	Clinical Policy Committee
Version	v1

1. Purpose and scope

This policy sets out the clinical criteria Bellcourt reviewers use to decide whether the service described in this policy (Advanced Imaging: MRI of the Lumbar Spine) is medically necessary. It is applied to prior authorization requests where the date of service falls within the effective period stated above. A client's plan document may exclude this service or impose additional requirements; plan provisions are checked first.

2. Definitions

Medically necessary: services that are appropriate and consistent with the diagnosis, in accordance with generally accepted standards of medical practice, and not primarily for the convenience of the member or provider.

3. Coverage criteria

The service is considered medically necessary when the criteria below are met, read together using the AND / OR connectors shown:

- 1 Presence of a red-flag condition (suspected cauda equina syndrome, suspected spinal infection, suspected malignancy, progressive neurological deficit, or significant trauma); OR
- 2 Radicular pain or radiculopathy that has persisted despite at least 6 weeks of documented conservative therapy (physical therapy, supervised exercise, NSAIDs or activity modification).

4. Not medically necessary / limitations

- Uncomplicated low back pain without radiculopathy and without red flags.
- Imaging requested solely for reassurance or at patient request.

5. Documentation requirements

Office notes documenting symptom duration, neurological exam, conservative therapy type and duration; surgeon's note if requested for pre-surgical planning.

If required documentation is missing, the request must be pended and the provider contacted for information. It must not be denied for lack of documentation until the information request has gone unanswered within the regulatory timeframe.

6. Version history

Version	Effective	Summary
v1	2024-01-01 to 2025-12-31	Initial version.
v2	2026-01-01	Conservative-therapy duration reduced from 6 weeks to 4 weeks; pre-surgical planning added as an indication. Approved by the Clinical Policy Committee on 2025-11-20.